

# **Comprehensive Clinical Cases**

## **Pediatric Case Proformas**

### **1. Gastrointestinal system.**

#### **Patient particulars.**

Name

Age

Gender

Education/schooling

Address

Date of admission

Date of examination

Informant

Reliability

Birth order

#### **Chief complaints:**

Vomiting

Nausea

Yellowish Discolouration of the eyes, skin

Decreased urine output

Distension of abdomen

Facial puffiness

Pain abdomen

Constipation

Fever

Diarrhoea  
Dysentery  
Melena  
Decreased appetite  
Hemetemesis

## **History of present illness.**

Pain abdomen:

Onset  
Duration  
Progression  
Site of pain  
Type  
Radiation  
Aggravating factor  
Relieving factors  
Associated symptoms

Vomiting:

Onset  
Duration  
Progression  
Associated with nausea/not  
Number of episodes  
Content  
Blood stained  
Foul smelling  
Associated symptoms like pain abdomen, fatigue, feeling of increased thirst, decreased urine output, decreased activity, apathy.  
Relation with food intake  
Bilious or non bilious

Aggravating factor

Relieving factors

Associated painful swallowing /difficulty in swallowing.

Fever :

Duration

Progression

Grade

Diurnal variation

Aggravating factor

Relieving factor

Associated symptoms like irritability,unconsolable crying, chills ,headache, nausea and vomiting, pain abdomen, burning micturition, yellowish discolouration

Diarrhoea and / Loose stools :

Onset

Duration

Progression

Colour

Blood tinged

Foul smelling

Nature of the stools

Number of episodes in a day

Apathy/Decreased activity

Food Intake

Type of Food taken recently

Time interval between food intake and onset of Diarrhoea

How is the child in symptom free period

Associated pain abdomen

Sensation of Incomplete evacuation of bowel

Nausea -vomiting

Fatigue

Increased thirst  
Aggregating factor  
Relieving factors

Bloody Diarrhoea:

Onset  
Duration  
Progression  
Aggravating factor  
Relieving factors  
Associated symptoms like pain abdomen  
Fever.

Constipation:

Onset  
Duration  
Passage of flatus  
Frequency of stools  
Altered consistency of the stools  
Difficulty in passing the stools  
Pain during defecation

Distension of abdomen:

Onset  
Duration  
Progression  
Aggravating factor  
Reliving factors  
Associated pain abdomen  
Decreased urine output  
Facial puffiness  
Swelling of the limbs

Generalised swelling of the body  
Yellowish Discolouration of skin and eyes  
Mass palpable per abdomen  
Associated fever and previous history of respiratory or akin infections.

Yellowish Discolouration (jaundice)

Onset

Duration

Progression

Associated Distension of abdomen

Itching

High coloured urine

Fever.

Pain abdomen

Palpable mass per abdomen

Nausea -vomiting.

Stool colour

Any aggravating factor & Reliving factors

Bleeding per rectum

Mass per rectum

Gastrointestinal bleed

Hemetemesis

Melena:

Hematochezia:

Onset

Duration

Progression

Frank blood or altered blood

Number of episodes

Associated Distension of abdomen

Pain abdomen.

Yellowish Discolouration of skin and eyes

Bleeding per rectum

Mass per rectum

H/o instrumentation

Bleeding disorder

Syncopal attacks

Foul smelling

Difficulty in swallowing and or painful swallowing :

Onset

Duration

Progression

On intake of liquid food and /solid food

Heart burn

Bad breath

Nausea -vomiting

Regurgitation symptoms.

Pain abdomen.

Burning micturition:

Onset

Duration

Progression

Pain abdomen

Fever

Painful urination

High coloured urine

Aggravating factor

Reliving factors

Decreased urine output :

Onset

Duration

Progression

Associated swelling of the limbs

Generalised swelling of the body

Headache

Body pain

Fatigue

Palpable abdominal lump:

Onset

Duration

Progression

Size

Site

Change in size

Swelling of the limbs:

Onset

Duration

Progression

First started in which place

Associated symptoms like

Decreased urine output

Yellowish Discolouration of skin and eyes

High coloured urine

Fever

Facial puffiness(early morning)

Generalised swelling.

Generalised swelling of the body

Onset

Duration

Progression

First started in which place

Associated symptoms like  
Decreased urine output  
Yellowish Discolouration of skin and eyes  
High coloured urine  
Fever  
Facial puffiness(early morning)  
Generalised swelling.  
H/o allergy  
Urticaria  
Drug intake.

Also ask h/o  
Skin changes  
Itching/purpura  
Upper abdominal discomfort  
Loss of appetite  
Loss of weight  
Apathy  
Decreased activity  
Altered behaviour  
Early morning convulsions  
Joint pain/swelling  
Passing of worms in stools

### **Past history.**

Similar complaints in the past  
Jaundice  
Gastrointestinal bleeds passage of worms  
Tuberculosis  
Blood transfusion/needle prick  
Contact with jaundice patient  
Sepsis

Preceding illness/hospitalization /surgery

## **Treatment history.**

## **Antenatal history:**

1st Trimester:

Birth order.

Pregnancy was confirmed after \_\_\_\_ of amenorrhea.

Urine pregnancy test was done or not.

H/o fever with rashes ,excessive vomiting, fatigue, exposure to radiation ,regular intake of drug, burning micturition ,increased frequency of micturition, bleeding or leaking per vagina .

Regular Antenatal visits.

Weight gain

Scan

Folic acid intake

2nd Trimester

Quickening felt in --month of gestation.

H/o headache, fever with rashes, any palpable swelling behind the ear,giddiness,swelling in the lower limbs which doesn't subside with rest, blurring of vision, bleeding or leaking Per vagina ,pain abdomen.

Iron and calcium supplements taken or not.

Regular ANC

Weight gain

Scan

3rd Trimester :

Fetal movement well perceived or not.

H/o bleeding Per vagina ,leaking per vagina ,pain abdomen, swelling in the lower limbs which doesn't subside with rest,burning micturition ,increased frequency of

micturition.

Iron and calcium intake

Regular Ante natal visits

Scan

Weight gain.

### **Natal history:**

LMP (Date of last menstrual period)

EDD (Expected date of delivery)

Birth order

Place of delivery.

Normal delivery or C-section

If C-section -Reason why C-section was done.

When did the Labour pain start.

H/o leaking PV

In she was brought to hospital

Describe in brief the events (admission-delivery the baby)/uneventful.

Gender of the baby

Birth weight of the baby

Baby cried immediately.

Baby kept in NICU

Breastfeeding initiated ? When?

### **Post natal history :**

Birth weight and gender

Baby cried immediately or not.

H/o delayed passage of urine or meconium

H/o yellowish discoloration of skin ,sclera.

H/o phototherapy.

Usage of NICU

Any diagnosed congenital anomalies

Breast feeding initiated /not.? If yes When?  
 H/o supplementary or complementary feeding.  
 Baby used to pass urine and stools regularly  
 Used to sleep for 2 to 3 hr after feeding.?  
 Breast feeding; how many times a day  
 How many times during night hours  
 Any usage of complementary feeding  
 Or inability to feed.

**Past obstetric history of mother :**

Married since how many years?  
 Consanguineous /non Consanguineous marriage.  
 Rx for infertility?  
 H/o abortion?  
 No.Children.  
 Describe each of them briefly as above.

**Diet/Nutritional history.**

| Timings   | Food item | Amount consumed | Calorie intake (kcal) | Protein in g |
|-----------|-----------|-----------------|-----------------------|--------------|
| Morning   |           |                 |                       |              |
| Afternoon |           |                 |                       |              |
| Evening   |           |                 |                       |              |
| Night     |           |                 |                       |              |
| Any other |           |                 |                       |              |

  

|                | Total intake | Expected | Deficit | Inference |
|----------------|--------------|----------|---------|-----------|
| Calorie intake |              |          |         |           |
| Protein intake |              |          |         |           |

**Developmental history.**

Explain the age at which successive milestones in each domain is attained.

- Gross motor
- Fine motor
- Language and communication
- Social and behavioural milestones.

Note for any developmental delay (if yes developmental assessment to be done in examination part)

### **Immunization History.**

| Name of the vaccine | Age at which it's given | Taken or not | Any adverse effects observed |
|---------------------|-------------------------|--------------|------------------------------|
|                     |                         |              |                              |

### **Family history**

Similar complaints in the family (if relevant)

Congenital anomalies etc

Pedigree chart

### **Socio-economic history.**

Head of the family

Per capita income.

Belongs to what (classification ..Kuppuswamy

Or BG Prasad classification)

### **Environmental history**

### **Summary after history**

### **General physical examination.**

Child is conscious ,playful and alert.

## Vitals.

Pulse rate

Respiration

Blood pressure.

Temperature.

Pallor

Icterus

Cyanosis

Clubbing

Koilonychia

Edema

Lymphadenopathy

## **Anthropometry.**

| Parameter                   | Observed | Expected | Inference. |
|-----------------------------|----------|----------|------------|
| Height /Length              |          |          |            |
| Weight                      |          |          |            |
| BMI (Body mass index)       |          |          |            |
| Chest circumference         |          |          |            |
| Head circumference.         |          |          |            |
| Upper segment/lower segment |          |          |            |
| Arm span                    |          |          |            |
| Mid arm circumference.      |          |          |            |

## Head toe examination

## Developmental assessment

| Mile stones                        | Age of attainment expected | Observed | Inference |
|------------------------------------|----------------------------|----------|-----------|
| Gross motor                        |                            |          |           |
| Fine motor                         |                            |          |           |
| Language and communication         |                            |          |           |
| Social and Behavioural mile stones |                            |          |           |

Developmental quotient.

## Systemic examination.

### Abdomen examination

Inspection:

Shape of the abdomen.

Distended or not

If distended -uniform or not

Umbilicus-central/everted/inverted

Flanks

Renal angle

Sinus /scar/dilated veins

Hernial orifices

Visible peristalsis

All regions move equally with respiration or not.

Visible lump

Palpation

All the inspectory findings should be confirmed

Shape of the abdomen

Symmetry

Local rise of temperature

Any Tenderness

Abdominal girth

Vertical Length (if distended)

Xiphisternum -umbilical distance

Umbilicus-pubic symphysis distance

Guarding/rigidity

Palpable lump

(If yes ,describe the lump in terms of

Size,Shape,surface, skin over the lump,consistency,temperature over the lump,fluctuation,surrounding area)

Renal angle- look for tenderness

Organomegaly -Palpation of spleen ,liver,Kidney.

Liver:

Surface

Consistency

Surface

Movement with respiration

Tenderness

Inability to insinuate finger between mass and coastal margin

Spleen:

Direction

Edge

Surface

Consistency

Surface

Movement with respiration

Tenderness

Inability to insulate finger between mass and coastal margin

Percussion

Tympanic note heard

Liver dullness

Tidal percussion

Fluid thrill

Shifting dullness

Percussion of spleen

Cardiac dullness

Auscultation

Bowel sounds heard or not

Venous hum

Arterial bruit

**Other system.**

Cardiovascular system

Respiratory system

Central nervous system

**Summary**

**Provisional diagnosis**

**Differential diagnosis**

**Investigations**

**Treatment**

## **2. Nutritional disorder.**

### **Patient particulars.**

Name

Age

Gender

Education/schooling

Address

Date of admission

Date of examination

Informant

Reliability

Birth order

### **Chief complaints:**

Inability to gain weight.

### **History of present illness**

(Describe in detail)

Vomiting

Diarrhoea

Abdominal Distension

Failure to thrive

Ear discharge

Skin lesions

Recurrent Respiratory tract infections

Oliguria, polyuria,

Burning micturition, hematuria

Loss appetite

Malnutrition

Jaundice

Steatorrhea

Ingestion of inedible things (pica)

Wasting of muscles

Decreased activity

Feeding difficulties

Swallowing problems

Dyspnoea, Breathlessness, palpitations.

History of contact with communicable diseases like tuberculosis

### **Past history.**

Exanthematous rashes

Fever

Tuberculosis

Whooping cough

Diarrhoea

Jaundice

Diagnosed congenital anomalies

### **Treatment history.**

Surgery

Hospitalization

### **Antenatal history :**

1st Trimester:

Birth order.

Pregnancy was confirmed after \_\_\_\_ of amenorrhea.

UPT was done or not.

H/o fever with rashes, excessive vomiting, fatigue, exposure to radiation, regular intake of drug, burning micturition, increased frequency of micturition, bleeding or leaking per vagina.

Regular ANC

Weight gain

Scan

Folic acid intake

2nd Trimester

Quickening felt in --month of gestation.

H/o headache, fever with rashes, any palpable swelling behind the ear, giddiness, swelling in the lower limbs which doesn't subside with rest, blurring of vision, bleeding or leaking per vagina, pain abdomen.

Iron and calcium supplements taken or not.

Regular Antenatal visits

Weight gain

Scan

3rd Trimester :

Fetal movement well perceived or not.

H/o bleeding per vagina, leaking pv, pain abdomen, swelling in the lower limbs which doesn't subside with rest, burning micturition, increased frequency of micturition.

Iron and calcium intake

Regular Ante natal visits

Scan

Weight gain.

### **Natal history:**

LMP (Date of Last menstrual period)

EDD (Expected date of delivery)

Birth order

Place of delivery.

Normal delivery or C-section

If C-section -Reason why C-section was done.

When did the Labour pain start.

H/o leaking PV

In she was brought to hospital

Describe in brief the events (admission-delivery the baby)/uneventful.

Gender of the baby

Birth weight of the baby

Baby cried immediately.

Baby kept in NICU

Breastfeeding initiated ? When?

### **Post natal history :**

Birth weight and gender

Baby cried immediately or not.

H/o delayed passage of urine or meconium

H/o yellowish discoloration of skin ,sclera.

H/o phototherapy.

Usage of NICU

Any diagnosed congenital anomalies

Breast feeding initiated /not.? If yes When?

H/o supplementary or complementary feeding.

Baby used to pass urine and stools regularly

Used to sleep for 2 to 3 hr after feeding.?

Breast feeding; how many times a day

How many times during night hours

Any usage of complementary feeding

Or inability to feed.

### **Past obstetric history of mother :**

Married since how many years?

Consanguineous /non Consanguineous marriage.

Rx for infertility?

H/o abortion?

No.Children.

Describe each of them briefly as above.

### **Diet/Nutritional history.**

Describe in detail what the child consumes at home in a day.

| Timings   | Food item | Amount consumed | Calorie intake (kcal) | Protein in g |
|-----------|-----------|-----------------|-----------------------|--------------|
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| Any other |           |                 |                       |              |

|                | Total intake | Expected | Deficit | Inference |
|----------------|--------------|----------|---------|-----------|
| Calorie intake |              |          |         |           |
| Protein intake |              |          |         |           |

### **Developmental history.**

Explain the age at which successive milestones in each domain is attained.

-Gross motor

-Fine motor

-Language and communication

-Social and behavioural milestones.

Note for any developmental delay (if yes developmental assessment to be done in examination part)

### **Immunization History.**

| Name of the vaccine | Age at which it's given | Taken or not | Any adverse effects observed |
|---------------------|-------------------------|--------------|------------------------------|
|                     |                         |              |                              |

## **Family history**

Similar complaints in the family (if relevant)

Congenital anomalies etc

Diabetes mellitus, hypertension, Bronchial asthma, Tuberculosis, Malignancies in the family.

## **Socio-economic history.**

Head of the family

Per capita income.

Belongs to what (classification ..Kuppuswamy

Or BG Prasad classification)

## **Environmental history**

## **Summary after history**

## **General physical examination.**

Child is conscious, playful and alert.

Vitals.

Pulse rate

Respiration

Blood pressure.

Temperature.

Pallor  
Icterus  
Cyanosis  
Clubbing  
Koilonychia  
Edema  
Lymphadenopathy

### **Anthropometry.**

| Parameter                   | Observed | Expected | Inference. |
|-----------------------------|----------|----------|------------|
| Height /Length              |          |          |            |
| Weight                      |          |          |            |
| BMI (Body mass index)       |          |          |            |
| Chest circumference         |          |          |            |
| Head circumference.         |          |          |            |
| Upper segment/lower segment |          |          |            |
| Arm span                    |          |          |            |
| Mid arm circumference.      |          |          |            |

### **Head toe examination**

### **Developmental assessment**

| Mile stones                        | Age of attainment expected | Observed | Inference |
|------------------------------------|----------------------------|----------|-----------|
| Gross motor                        |                            |          |           |
| Fine motor                         |                            |          |           |
| Language and communication         |                            |          |           |
| Social and Behavioural mile stones |                            |          |           |

Developmental quotient

## **Systemic examination.**

Cardiovascular system

Respiratory system

Abdomen examination

Central nervous system examination

## **Summary**

## **Provisional diagnosis**

## **Differential diagnosis**

## **Investigations**

## **Treatment**

### **3. New born**

#### **Patient Particulars**

Baby of?

Date of Birth

Age

Gender

Gestational age

Last date of menstruation of the mother

Expected date of delivery

Delivered on

Weight of the baby

Type: Appropriate for gestational age or not

Baby is in early/late neonatal period

Place and mode of delivery

Address of the mother

Age of the mother

Parity index

Informant

Reliability

#### **Chief complaints**

Delayed /not passed meconium or urine

Yellowish Discolouration of the skin ,eyes

Fever

Involuntary movements etc

#### **History of present illness.**

Onset

Duration

Progression

Associated symptoms

Aggravating factor

Relieving factors

Admission to Neonatal Intensive Care Unit

Treatment history

## **Antenatal history :**

1st Trimester:

Birth order.

Pregnancy was confirmed after \_\_\_\_ of amenorrhea.

UPT was done or not.

H/o fever with rashes ,excessive vomiting, fatigue, exposure to radiation ,regular intake of drug, burning micturition ,increased frequency of micturition, bleeding or leaking per vagina.

Regular ANC

Weight gain

Scan

Folic acid intake

2nd Trimester

Quickening felt in --month of gestation.

H/o headache, fever with rashes, any palpable swelling behind the ear,giddiness,swelling in the lower limbs which doesn't subside with rest, blurring of vision, bleeding or leaking Per vagina,pain abdomen.

Iron and calcium supplements taken or not.

Regular ANC

Weight gain

Scan

3rd Trimester :

Fetal movement well perceived or not.

H/o bleeding Per vagina ,leaking per vagina ,pain abdomen,swelling in the lower limbs which doesn't subside with rest,burning micturition ,increased frequency of micturition.

Iron and calcium intake

Regular ANC

Scan

Weight gain.

### **Natal history:**

LMP (Date of last menstruation)

EDD (Expected date of delivery)

Birth order

Place of delivery.

Normal delivery or C-section

If C-section -Reason why C-section was done.

When did the Labour pain start.

H/o leaking PV

In she was brought to hospital

Describe in brief the events (admission-delivery the baby)/uneventful.

Gender of the baby

Birth weight of the baby

Baby cried immediately.

Baby kept in NICU

Breastfeeding initiated ? When?

### **Post natal history :**

Birth weight and gender

Baby cried immediately or not.

H/o delayed passage of urine or meconium

H/o yellowish discoloration of skin ,sclera.

H/o phototherapy.

Usage of NICU

Any diagnosed congenital anomalies

### **Breastfeeding/Feeding history.**

Breast feeding initiated /not.?

If yes When?

How many times a day

How many times during night hours

Supplementary or complementary feeding.

Baby used to pass urine and stools regularly

Used to sleep for 2 to 3 hr after feeding.?

Inability to feed.

Any complications

### **Immunization History.**

### **Maternal history.**

Past obstetric history of mother :

Married since how many years?

Consanguineous /non Consanguineous marriage.

Rx for infertility?

H/o abortion?

No.Children.

Describe each of them briefly as above.

### **Past history**

### **Family history**

### **Socio-economic history**

## **Environmental history.**

## **Summary after history.**

## **General physical examination.(of the baby)**

Posture/Attitude

Activity and general behaviour

Cyanosis

Icterus

Assessment of gestational age: New modified Ballard scoring

Congenital defects

## **Vitals**

Temperature

Respiratory rate

Heart rate

Blood pressure

## **Anthropometry**

Birth weight

Length

Head circumference

Chest circumference

Ponderal index

Inference

## **Head to toe examination.**

Head

Moulding

Caput succedaneum

Anterior and posterior fontanelle

Micro/macro cephal

Any bruises

Encephelocoel

Face

Dysmorphic facies

Eyes

Ears

Nose

Oral cavity

Neck

Chest

Umbilical cord and umbilicus

Genitals

Back

Hips

Extremities-limbs ,digits,palm,sole,nails

Skin examination

Oedema

Skeletal system

Orifice counting and test for patency

Look for congenital anomalies

## **Systemic examination**

Respiratory system examination

Abdomen examination

Central nervous system examination with neonatal reflexes

Cardiovascular system examination

## **Summary**

## **Management**

## **4. Central nervous system.**

### **Patient particulars .**

Name

Age

Gender

Education/schooling

Address

Date of admission

Date of examination

Informant

Reliability

Birth order

### **Chief complaints:**

Altered sensorium

Inability to use the limbs, paucity of the movements. Etc

### **History of present illness.**

Onset:

Acute

Sub acute

Chronic

Paroxysmal

Precipitating factor.

When did it start

What was the child doing when it started.(playing/working/ related to sleep)

Course of illness.

Progressive/Static/improving

Associated complaints.

Fever

Convulsion

History related to higher mental functions.

Level of consciousness:

Recognition of father/mother

Unconsciousness:

Onset

Duration

Progression

Number of attacks

Duration of unconsciousness

Associated symptoms

Loss of orientation

Behavioural changes

Cognitive functions :speech, language.

Child is playing well,eating,associating,sleeping well.

Loss of memory.

Emotional disturbances.

History related to sleep.

History of handedness.

History related to cranial nerve involvement

Cranial nerve symptoms (complaints pertaining to cranial nerve dysfunction)

Disturbance in perception of smell

Visual dysfunction

Diplopia, Squint

Difficulty in eating

Dribbling of saliva

Deviation of angle of mouth

Collection of food in the mouth

Inability to close the eyes

Decreased taste sensation

Hearing disturbances ,Vertigo.

Nasal twang to speech , nasal regurgitation of food .

Difficulty in shrugging of the shoulder

Dysphagia

History related to motor system.

Wasting/thing of muscle

Stiffness /flaccidity

Weakness:onset,duration,progression,part involved (right/left)

Upper limb:

Distal-difficulty in buttoning etc

Proximal-difficulty in taking bath etc

Lower limb:

Distal- Difficulty in gripping the sandals etc

Proximal-Difficulty in squatting etc

History related to stance and gait.

History related to Sensory system.

Onset

Duration

Progression

Parts of the body involved:

Superficial: Touch ,temperature, pain

Deep: Crude touch, pressure, position and joint sensation, vibration sense.  
Sensation: total loss, partial loss, increased sensation, altered sensations (tingling and numbness).

History related to cerebellar symptoms.

Hypotonia

Ataxia

Titubation

Nystagmus

Staccato speech

Intention tremor

Change in the handwriting

Ataxic gait

Inability to perform alternating movements

Swaying while walking.

History related to autonomic system

Bladder and bowel disturbances

Vasomotor changes like flushing, sweating.

Giddiness on getting up from the bed

Palpitation

Salivary disturbances

History related to spine and cranium

Pain in the back

Spinal deformities

Painful movement of the spine

Increase or decrease in the head size

Also ask, h/o

Headache

Fever

Vomiting

Visual disturbances  
Convulsion/coma  
Diarrhoea/dehydration  
Visual disturbances  
Trauma  
Ear pain, ear discharge, exanthematous rashes.  
Altered sensorium  
Painful movement of the neck  
Palpitations  
Cyanosis  
Cough  
Foul smelling sputum  
Orthopnoea  
Abnormal breathing  
Bleeding tendency

History related to cerebral palsy

### **Past history**

Similar illness in the past.  
Convulsion  
Head injury  
Ear infection  
Rheumatic fever  
Any other Diagnosed illnesses

### **Treatment history.**

Surgery  
Hospitalization

### **Antenatal history :**

1st Trimester:

Birth order.

Pregnancy was confirmed after \_\_\_\_ of amenorrhea.

UPT was done or not.

H/o fever with rashes ,excessive vomiting, fatigue, exposure to radiation ,regular intake of drug, burning micturition ,increased frequency of micturition, bleeding or leaking pv.

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Weight gain

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Iron and calcium supplements taken or not.

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3rd Trimester :

Fetal movement well perceived or not.

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Iron and calcium intake

Regular ANC

Scan

Weight gain.

**Natal history:**

LMP

EDD

Birth order

Place of delivery.

Normal delivery or C-section

If C-section -Reason why C-section was done.

When did the Labour pain start.

H/o leaking PV

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Describe in brief the events (admission-delivery the baby)/uneventful.

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Breastfeeding initiated ? When?

**Post natal history:**

Birth weight and gender

Baby cried immediately or not.

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H/o phototherapy.

Usage of NICU

Any diagnosed congenital anomalies

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Consanguineous /non Consanguineous marriage.

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Describe each of them briefly as above.

### **Diet/Nutritional history.**

Describe in detail what the child consumes at home in a day

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| Afternoon |           |                 |                       |              |
| Evening   |           |                 |                       |              |
| Night     |           |                 |                       |              |
| Any other |           |                 |                       |              |

|                | Total intake | Expected | Deficit | Inference |
|----------------|--------------|----------|---------|-----------|
| Calorie intake |              |          |         |           |
| Protein intake |              |          |         |           |

### **Developmental history.**

Explain the age at which successive milestones in each domain is attained.

-Gross motor

-Fine motor

-Language and communication

-Social and behavioural milestones.

Note for any developmental delay (if yes developmental assessment to be done in examination part)

### **Immunization History.**

| Name of the vaccine | Age at which it's given | Taken or not | Any adverse effects observed |
|---------------------|-------------------------|--------------|------------------------------|
|                     |                         |              |                              |

### **Family history**

Similar complaints in the family (if relevant)

Congenital anomalies etc

Diabetes mellitus, asthma, tuberculosis, hypertension, Malignancies.

Pedigree chart

### **Socio-economic history.**

Head of the family

Per capita income.

Belongs to what (classification ..Kuppuswamy

Or BG Prasad classification)

### **Environmental history**

### **Summary after history**

### **General physical examination.**

Child is conscious, playful and alert.

Vitals.

Pulse rate

Respiration

Blood pressure.

Temperature.

Pallor

Icterus

Cyanosis

Clubbing

Koilonychia

Edema

Lymphadenopathy

Anthropometry.

| Parameter                   | Observed | Expected | Inference. |
|-----------------------------|----------|----------|------------|
| Height /Length              |          |          |            |
| Weight                      |          |          |            |
| BMI (Body mass index)       |          |          |            |
| Chest circumference         |          |          |            |
| Head circumference.         |          |          |            |
| Upper segment/lower segment |          |          |            |
| Arm span                    |          |          |            |
| Mid arm circumference.      |          |          |            |

**Head toe examination**

## Developmental assessment

| Mile stones                        | Age of attainment expected | Observed | Inference |
|------------------------------------|----------------------------|----------|-----------|
| Gross motor                        |                            |          |           |
| Fine motor                         |                            |          |           |
| Language and communication         |                            |          |           |
| Social and Behavioural mile stones |                            |          |           |

Developmental quotient :

## Systemic examination.

### CNS examination.

Higher mental function examination

Conscious

Operative

Orientation to Time,place,person.

Memory:Immediate,Recent,remote.

Intelligence.

Illusion/Delusion/Hallucination.

Speech

Cranial nerve examination.

1)Olfactory-1

Sensation of smell.

2)Optic-2

Visual acuity

Distant

Near

Colour vision

Visual field

Light reflex-direct and indirect

Accommodation reflex

3)3,4,6:Examination.

Eye ball examination.

On all the directions.

Pupil-position

Size

Shape

Symmetry

Ptosis

4)5th CN.

Sensory: sensations over the face.

Motor: clenching of teeth

Lateral movement of jaw

Reflex: cornealreflex, jaw jerk

5)7thCN.

Forehead frowning

Eyebrow raising

Eye closure.

Teeth showing

Blowing the cheek

Nasolabial fold.

Weak platysma

Taste sensation over anterior two- third of tongue .

6)8thCN.

Weber's test

Rinne's test

7)9th CN and 10th CN.

Movement of palate

Position of uvula

Taste sensation over posterior one-third of tongue

8)11th CN

Shrugging of shoulder

Turning neck against resistance.

9)12 th CN.

Protrusion of tongue

Fasciculation

Wasting

Deviation

Dysarthria

Motor system.

1.Attitude of the limbs.

Upper limbs

Lower limbs.

2.Nutrition.

Upper limbs

Arm

Forearm.

Lower limbs.

Thigh

Leg

### 3.Tone

Upper limbs

Hypotonic

Hypertonia

-Rigidity

-Spasticity

Lower limbs

Hypotonia

Hypertonia

-Rigidity

-Spasticity

### 4)Power

(Mention the grade)

Upper limbs

Shoulder:

Adduction

Abduction

Flexion

Extension

Elbow

Flexion

Extension

Wrist

Flexion

Extension

Lower limbs

Hip

Adduction

Abduction

Flexion

Extension

Knee

Flexion

Extension

Ankle

Dorsiflexion

Plantarflexion.

5)Co-ordination.

Upper limb: Finger nose test

Finger nose finger test

Drawing circle

Lower limbs.

Drawing circle

Knee heel test.

6)Gait abnormality if any.

7)Reflexes.

Superficial reflexes.

Corneal reflex

Abdominal reflex

Plantar reflex

Babinski sign

Cremasteric reflex

Biceps

Triceps

Supinator reflex

Knee

Ankle

Clonus: plantar and ankle

Sensory system.

1)superficial

Touch

Temperature

Pain

2)Deep

Pressure

Crude touch

Vibration

Joint sensation

Position sensation.

3)Cortical sensations.

Tactile localisation

Tactile discrimination

Tactile extinction.

Stereognosis

Graphaesthesia

Cerebellar system examination.

Nystagmus

Scanning of speech

Hypotonia

Intentional tremor

Dysdiadokokinesis

Rebound phenomenon

Titubation

Knee heel test

Pendular knee jerk.

Tandom walking

Cerebellar gait.

Skull and spine

Meningeal signs.

Neck stiffness

Kernig's Sign

Brudzinski's sign

-Leg sign

-Neck sign

Straight leg raising test.

### **Other system.**

CVS

RS

Abdomen

### **Summary**

### **Provisional diagnosis**

### **Differential diagnosis**

### **Investigations**

### **Treatment**

## **5. Respiratory system**

### **Patient Particulars**

Name

Age

Gender

Education/schooling

Address

Date of admission

Date of examination

Informant

Reliability

Birth order

### **Chief complaints**

Fever with chills

Cough

Post tussive vomiting

Noisy breathing

Wheeze

Breathlessness

Hurried breathing

### **History of present illness.**

Fever:

Duration

Progression

Grade

Type

Chills

Headache

Associated symptoms

Diurnal variation

Seasonal variation.

Cough:

Onset

Duration

Progression

Sleep disturbance

Posttussive vomiting

Diurnal variation ,

Postural variation,

Seasonal variation ,

Inability to take food,

Aggravating factor

Relieving factor

Posttussive vomiting:

Number of episodes

Onset

Since how long it's there

Content of the vomitus

Blood stained

Foul smelling

Breathlessness:

Onset

Duration

Progression

Associated wheeze or noisy breathing

Aggravating factor

Relieving factor  
Diurnal variation  
Postural variation  
Seasonal variation.

Hurried breathing.:  
Onset  
Duration  
Progression  
Associated symptoms like cough ,Breathlessness  
Aggravating factor  
Relieving factor  
Grunting.

Noisy breathing:  
Onset  
Duration  
Progression  
Aggravating factor  
Relieving factor  
Associated symptoms-grunting ,wheeze ,Cough.

Wheeze.:  
Onset  
Duration  
Progression  
Aggravating factor  
Relieving factor  
Diurnal variation  
Seasonal variation  
Associated symptoms :cough

Grunting:

Onset,  
Duration ,  
Progression  
AF or RF

Cyanosis (blueish discoloration of periphery)  
Syncope attacks  
Decreased food intake  
Inability to sleep  
Irritability  
Inconsolable crying  
Running nose  
Ear pain  
Ear discharge  
Nasal blockage  
Mouth breathing  
Headache (older child)  
Hoarseness of voice  
Exposure to smoke ,dust ,indoor pollutants,  
h/o allergy  
H/o bottle feeding.  
H/o diagnosed congenital anomalies

**Past history:**

Similar complaints in the past.  
Congenital anomalies  
Treatment history

**Antenatal history :**

1st Trimester  
Birth order.  
Pregnancy was confirmed after \_\_\_\_ of amenorrhea.

UPT was done or not.

H/o fever with rashes ,excessive vomiting, fatigue, exposure to radiation ,regular intake of drug, burning micturition ,increased frequency of micturition, bleeding or leaking per vagina.

Regular Antenatal visits.

Weight gain

Scan;

Folic acid intake

## 2nd Trimester

Quickening felt in --month of gestation.

H/o headache, fever with rashes, any palpable swelling behind the ear,giddiness,swelling in the lower limbs which doesn't subside with rest, blurring of vision, bleeding or leaking per vagina ,pain abdomen.

Iron and calcium supplements taken or not.

Regular ANC

Weight gain

Scan

## 3rd Trimester

Fetal movement well perceived or not.

H/o bleeding per vagina ,leaking per vagina ,pain abdomen, swelling in the lower limbs which doesn't subside with rest, burning micturition ,increased frequency of micturition.

Iron and calcium intake

Regular ANC

Scan

Weight gain.

## **Natal history:**

LMP

EDD

Birth order

Place of delivery.

Normal delivery or C-section

If C-section -Reason why C-section was done.

When did the Labour pain start.

H/o leaking PV

In she was brought to hospital

Describe in brief the events (admission-delivery the baby)/uneventful.

Gender of the baby

Birth weight of the baby

Baby cried immediately.

Baby kept in NICU

Breastfeeding initiated ? When?

## **Post natal history**

Birth weight and gender

Baby cried immediately or not.

H/o delayed passage of urine or meconium

H/o yellowish discoloration of skin ,sclera.

H/o phototherapy.

Usage of NICU

Any diagnosed congenital anomalies

Breast feeding initiated /not.? If yes When?

H/o supplementary or complementary feeding.

Baby used to pass urine and stools regularly

Used to sleep for 2 to 3 hr after feeding.?

Breast feeding; how many times a day

How many times during night hours

Any usage of complementary feeding

Or inability to feed.

## **Past obstetric history of mother**

Married since how many years?

Consanguineous /non Consanguineous marriage.

Rx for infertility?

H/o abortion?

Number of Children.

Describe each of them briefly as above.

Past history:

Any h/o similar complaints in the past.

Past treatment h/o.

Exposure to infectious diseases.

Any h/o surgical or medical interventions in the past.

### **Diet/Nutritional history.**

Describe in detail what the child consumes at home in a day.

| Timings   | Food item | Amount consumed | Calorie intake (kcal) | Protein in g |
|-----------|-----------|-----------------|-----------------------|--------------|
| Morning   |           |                 |                       |              |
| Afternoon |           |                 |                       |              |
| Evening   |           |                 |                       |              |
| Night     |           |                 |                       |              |
| Any other |           |                 |                       |              |

|                | Total intake | Expected | Deficit | Inference |
|----------------|--------------|----------|---------|-----------|
| Calorie intake |              |          |         |           |
| Protein intake |              |          |         |           |

### **Developmental history.**

Explain the age at which successive milestones in each domain is attained.

-Gross motor

-Fine motor

-Language and communication

-Social and behavioural milestones.

Note for any developmental delay (if yes developmental assessment to be done in examination part)

### **Immunization history.**

| Name of the vaccine | Age at which it's given | Taken or not | Any adverse effects observed |
|---------------------|-------------------------|--------------|------------------------------|
|                     |                         |              |                              |

### **Family history**

Similar complaints in the family (if relevant)

Congenital anomalies etc

Diabetes mellitus, asthma, tuberculosis, hypertension, Malignancies.

Pedigree chart

### **Socio-economic history.**

Head of the family

Per capita income.

Belongs to what (classification ..Kuppuswamy

Or BG Prasad classification)

### **Environmental history**

### **General physical examination.**

Child is conscious, playful and alert.

Vitals.

Pulse rate

Respiration

Blood pressure.

Temperature.

Pallor

Icterus  
 Cyanosis  
 Clubbing  
 Koilonychia  
 Edema  
 Lymphadenopathy

### Anthropometry.

| Parameter                   | Observed | Expected | Inference. |
|-----------------------------|----------|----------|------------|
| Height /Length              |          |          |            |
| Weight                      |          |          |            |
| BMI (Body mass index)       |          |          |            |
| Chest circumference         |          |          |            |
| Head circumference.         |          |          |            |
| Upper segment/lower segment |          |          |            |
| Arm span                    |          |          |            |
| Mid arm circumference.      |          |          |            |

### Head toe examination

### Developmental assessment .

| Mile stones | Age of attainment expected | Observed | Inference |
|-------------|----------------------------|----------|-----------|
| Gross motor |                            |          |           |

|                                   |  |  |  |
|-----------------------------------|--|--|--|
| Fine motor                        |  |  |  |
| Language and communication        |  |  |  |
| Social and Behavioural milestones |  |  |  |

Developmental quotient

## **Systemic examination.**

### **Respiratory system:**

Upper airway.

Nose

Throat.

Ear

Oral cavity

Inspection.:

Shape of the chest

Symmetry

Position of trachea

Apex beat

Chest movement in all the areas equal/not

(Supra clavicular,infra clavicular mammary,inframammary,axillary ,  
infraaxillary ,supra scapular ,infrascapular,interscapular)

Dilated veins ,scars ,sinuses

Crowding of ribs

Supra clavicular hollowing

Infra clavicular flattening

Drooping of shoulder

usage of accessory muscles of respiration.

Alar flaring

Intercostal in drawing

Palpation.: All the inspectory findings should be confirmed

Local rise of temperature

Tenderness

Position of trachea.

Apex beat

JVP

Chest movements

Chest measurement: Antero-posterior ,Transverse.

Chest expansion. Inspiration-expiration.

Tactile vocal fremitus.

(Supra clavicular,intra clavicular mammary,inframammary,axillary ,infraaxillary ,suprascapular ,infrascapular,interscapular)

Hemi thorax

Hemi thorax expansion

Spino- acromion distance

Percussion:

Resonant or impaired note

Liver dullness

Tidal percussion

Auscultation. (Supra clavicular,intra clavicular mammary,inframammary,axillary ,infraaxillary ,suprascapular ,infrascapular,interscapular)

Air entry

Intensity

Type of breath sounds heard

Added sounds

Vocal resonance

**Summary**

**Differential diagnosis**

**Provisional diagnosis**

**Investigation**

**Treatment**

## **6. Pediatrics-Cardiovascular system.**

### **Patient particulars .**

Name

Age

Gender

Education/schooling

Address

Date of admission

Date of examination

Informant

Reliability

Birth order

### **Chief complaints:**

Cough

Fever

Difficulty in breathing

Increased precordial activity

Failure to thrive  
Cyanosis  
Recurrent lower respiratory tract infections  
Joint pains  
-In older children(also ask)  
Cyanosis  
Exercise intolerance  
Difficulty in breathing  
Chest pain

### **History of present illness.**

Cough  
onset  
  
Duration  
Progression  
With /without post tussive vomiting  
Aggravating factors  
Relieving factors  
Diurnal variation  
Seasonal variation  
Positional variation.

Difficulty in breathing.  
Onset  
Duration,  
Progression,(grading),  
Orthopnoea,  
Platypnea,  
Trepopnea, paroxysmal nocturnal dyspnoea,  
Chest pain,  
Associated wheeze,

Diurnal /Seasonal /postural variation  
Aggravating factors ,Relieving factors.

Palpitation(older children):

Onset

Duration

Progression

Number of episodes

How often

Aggregating factors

Relieving factors

Associated symptoms like Chest pain.

Syncopal attacks:

Onset

Duration

Progression

At rest/exertion

Aggregating factors and relieving factors

Number of episodes, associated symptoms.

Chest pain(older children):

onset

Duration

Progression

Site

Character

Radiation

Aggravating factors

Relieving factors.

Recurrent Respiratory tract infections :

Number of episodes in a month

Severity

Associated fever

Hospitalization And treatment.

Oedema:

Onset

Duration

Progression

Where did it start first

Facial puffiness

Decreased urine output.

Constipation/Obstipation

Pain abdomen

Distended abdomen

Also ask h/o:

Irritability

Haemoptysis(older children)

Oliguria

Fatigue/Tiredness

Cyanosis

Specific Disease aetiology :

Rheumatic fever:

Sore throat

Fever

Epistaxis

Fleeting type of Joint pain and joint swelling.

Chest pain

Involuntary movements

Cyanotic heart disease:

Cyanosis

Cyanotic spells

Squatting/squatting equivalents

Increase in severity of cyanosis

Difficulty in breathing

Complications of Heart disease :

- \*Failure to thrive

- \*Symptoms of cardiac failure-Chest pain,edema,breathlessness,oliguria,pain abdomen

- \*Symptoms of infective endocarditis

- \*Thromboembolic manifestations-convulsions,Syncopalattacks,Visual disturbances

- \*Oliguria and nocturia

- \*Left atrial enlargement -Hoarseness of voice

- \*Fever

- \*Plethora

- \*Bleeding spots

- \*Increased crying

- \*Altered sensorium

- \*Convulsion

- \*Headache

- \*Pain abdomen

## **Past history.**

Similar complaints in the past.

Diagnosed congenital anomalies

Cardiac diseases.

Past treatment

Exposure to infectious diseases.

Any h/o surgical or medical interventions in the past.

## **Antenatal history :**

1st Trimester:

Birth order.

Pregnancy was confirmed after \_\_\_\_ of amenorrhea.

UPT was done or not.

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Regular Antenatal visits.

Weight gain

Scan

Folic acid intake

2nd Trimester

Quickening felt in --month of gestation.

H/o headache, fever with rashes, any palpable swelling behind the ear, giddiness, swelling in the lower limbs which doesn't subside with rest, blurring of vision, bleeding or leaking per vagina ,pain abdomen.

Iron and calcium supplements taken or not.

Regular ANC

Weight gain

Scan

3rd Trimester :

Fetal movement well perceived or not.

H/o bleeding Per vagina ,leaking per vagina ,pain abdomen, swelling in the lower limbs which doesn't subside with rest, burning micturition ,increased frequency of micturition.

Iron and calcium intake

Regular ANC

Scan

Weight gain.

### **Natal history:**

LMP (Date of last menstruation)

EDD (Expected date of delivery)

Birth order

Place of delivery.

Normal delivery or C-section

If C-section -Reason why C-section was done.

When did the Labour pain start.

H/o leaking PV

In she was brought to hospital

Describe in brief the events (admission-delivery the baby)/uneventful.

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H/o phototherapy.

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How many times during night hours  
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|                | Total intake | Expected | Deficit | Inference |
|----------------|--------------|----------|---------|-----------|
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Similar complaints in the family (if relevant)

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Head of the family

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Belongs to what (classification ..Kuppuswamy

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### **Summary after history**

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## Vitals.

Pulse rate

Respiration

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Temperature.

Pallor

Icterus

Cyanosis

Clubbing

Koilonychia

Oedema

Lymphadenopathy

## Anthropometry.

| Parameter                   | Observed | Expected | Inference. |
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| Chest circumference         |          |          |            |
| Head circumference.         |          |          |            |
| Upper segment/lower segment |          |          |            |
| Arm span                    |          |          |            |

|                        |  |  |  |
|------------------------|--|--|--|
| Mid arm circumference. |  |  |  |
|------------------------|--|--|--|

## Head toe examination

### Developmental assessment

| Mile stones                        | Age of attainment expected | Observed | Inference |
|------------------------------------|----------------------------|----------|-----------|
| Gross motor                        |                            |          |           |
| Fine motor                         |                            |          |           |
| Language and communication         |                            |          |           |
| Social and Behavioural mile stones |                            |          |           |

Developmental quotient

Systemic examination.

Cardiovascular system examination.

Inspection:

Visible precordial bulge

Visible apex impulse

Visible left parasternal pulsation.

Visible left 2nd ICS pulsation.

Visible suprasternal pulsation.

Any other visible pulsation.

Any scars ,sinuses, dilated veins.

Any bony abnormalities:kyphosis,scoliosis.

## Palpation.

Apex beat: confirmed to be

Localised or not

Character

Palpable site.

Palpable left parasternal heave.

Palpable epigastric pulsation.

Palpable 2nd intercostal space pulsation.

Any other palpable sounds or thrills.

Venous hum at the base of the neck.

Carotid thrill /pulsation.

JVP.

## Percussion.

Right heart border corresponds to right external border.

Left heart border corresponds to apex beat

Lower dullness

Hepatic span.

## Auscultation. (1<sup>st</sup>, 2<sup>nd</sup>, 3<sup>rd</sup>, 4<sup>th</sup> heart sounds and murmurs )

Mitral

Tricuspid

Aortic

Pulmonary

Other areas.

Describe the murmur

Type

Character

Radiation.

Best heard-position of the patient, phase of respiration.

Other systems.

Central nervous system

Abdomen

Respiratory system

## **Summary**

**Provisional diagnosis**

**Differential diagnosis**

**Investigations**

**Treatment**

**Follow-up**

**Prognosis**